

SPECIAL REPORT



**BRAIN HEALTH IN AMERICA:
UNDERSTANDING AND SUPPORTING
LIFELONG COGNITIVE HEALTH**

As more Americans live into their late 70s and beyond, public interest in proactively building and maintaining brain health is growing.¹⁰⁴³ But brain health (how well we think, learn and remember) is relevant long before the later stages of adulthood. There is increasing recognition that these cognitive functions are shaped throughout life. This year's Special Report offers new insight into how Americans understand, prioritize and act on the desire to sustain brain health across the lifespan.

Supporting Brain Health by Acting on Modifiable Factors With Lifestyle Choices and Behaviors

Cognitive decline, mild cognitive impairment (MCI) due to Alzheimer's disease, Alzheimer's disease and other causes of dementia are not an inevitable part of aging. We are learning more and more about the impact of non-modifiable risk factors such as age, genetics and family history on brain health. Researchers have also identified numerous modifiable factors with strong evidence connecting them to lower risk of cognitive decline and dementia (see Figure 2, page 17). These are reflected in the Alzheimer's Association *10 Healthy Habits for Your Brain*, which highlights key areas that influence cognitive well-being (Figure 19)¹⁰⁴⁴:

1. Traumatic brain injury.
2. Smoking.
3. Physical activity.
4. Cognitive engagement.
5. Hypertension (high blood pressure).
6. Diabetes.
7. Sleep.
8. Formal education.
9. Diet/nutrition.
10. Weight.

These healthy habits are likely familiar, as most overlap with recommendations for improving and maintaining physical health — such as controlling blood pressure, managing blood sugar, maintaining a healthy weight, eating a nutritious diet, staying physically active, getting

It's never too early — or too late — to start prioritizing brain health

Public attention and scientific research often concentrate on disorders affecting cognitive function in older adults, such as MCI due to Alzheimer's disease, Alzheimer's disease and other dementias. While most of the 7.4 million individuals living with clinical Alzheimer's dementia and the estimated 4.2 million Americans with MCI due to Alzheimer's are 65 and older, research shows that physiological changes marking the earliest stages of Alzheimer's disease can begin up to 20 years before measurable cognitive impairment.¹³⁻¹⁵ Because these early changes may be influenced, in part, by lifestyle behaviors during younger years, the benefits of nurturing brain health are not just for older adults, but important across the entire lifespan.

enough restful sleep and avoiding smoking. Cognitive engagement is about keeping mentally active through new and challenging activities (e.g., taking classes, learning new skills or a new language, reading difficult material or learning to play a challenging strategy game).

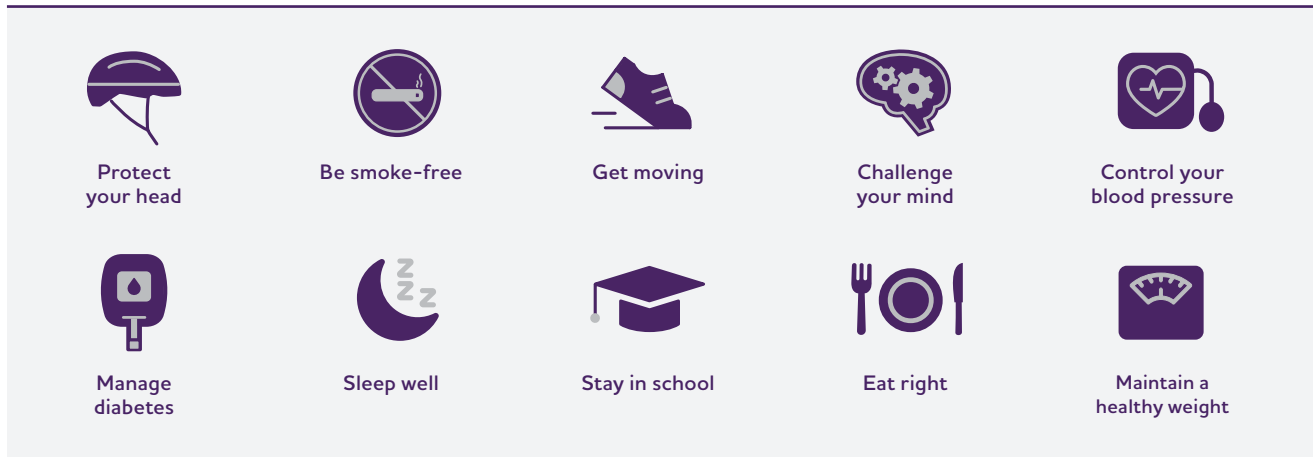
One notable report estimated that nearly half of all dementia cases globally could be attributed to modifiable risk factors.⁶⁵ Knowing these factors can help people take meaningful, practical steps to reduce their risk of cognitive decline and support cognitive health across the lifespan. Adopting healthy lifestyle habits may also meaningfully influence both short-term and long-term physical health. Some of these lifestyle behaviors are relatively straightforward to adopt, while others, such as managing chronic conditions or engaging in cognitively-stimulating activities, are multifaceted and may require support from a physician or a structured program.

The U.S. POINTER Study Substantiated the Power of Lifestyle Changes to Impact Brain Health

One of the strongest examples of how lifestyle changes affect brain health comes from the U.S. Study to Protect Brain Health Through Lifestyle Intervention to Reduce Risk

FIGURE 19

10 Healthy Habits for Your Brain



(U.S. POINTER). U.S. POINTER is the first large-scale, randomized controlled clinical trial in the United States to demonstrate that an accessible and sustainable healthy lifestyle intervention that addresses multiple factors simultaneously can protect cognitive function.²⁶⁸ The study evaluated a multi-domain “Brain Health Recipe” (Figure 20) of lifestyle changes targeting four modifiable risk factors associated with cognitive decline:

- Physical exercise.
- Cognitive exercise.
- Cardiovascular health monitoring.
- Nutrition.

Over two years, the multi-site trial randomized more than 2,100 study participants at elevated risk for cognitive decline to either a structured program that incorporated the brain health recipe shown in Figure 20 or a self-guided intervention in which individuals made their own plans focusing on the same four lifestyle factors.^{268,1045} The structured intervention included¹⁰⁴⁵:

- 38 facilitated peer group sessions that also provided an opportunity for social engagement.
- Prescribed aerobic, resistance and flexibility exercise with measurable goals.
- Adherence to the MIND diet (Mediterranean-DASH Intervention for Neurodegenerative Delay, a combination of a Mediterranean diet and the Dietary Approaches to Stop Hypertension [DASH] diet).
- Cognitive training.
- Semiannual clinical reviews of cardiovascular risk markers (cholesterol, hemoglobin A_{1c} and blood pressure) with goal setting.

The self-guided group received educational materials, attended six peer meetings over two years, and underwent annual health monitoring without individualized coaching.¹⁰⁴⁵ Participants in the study largely reflected the racial and ethnic diversity of the U.S. population.¹⁰⁴⁶

Both groups had improved cognitive function at the end of two years, with the structured group having a statistically-significant greater improvement in cognition compared with the self-guided group.²⁶⁸ Compared with the self-guided group, those in the structured program had cognitive scores equivalent to those of people who were up to two years younger.²⁶⁸

Midlife Is an Important Window for Long-Term Brain Health

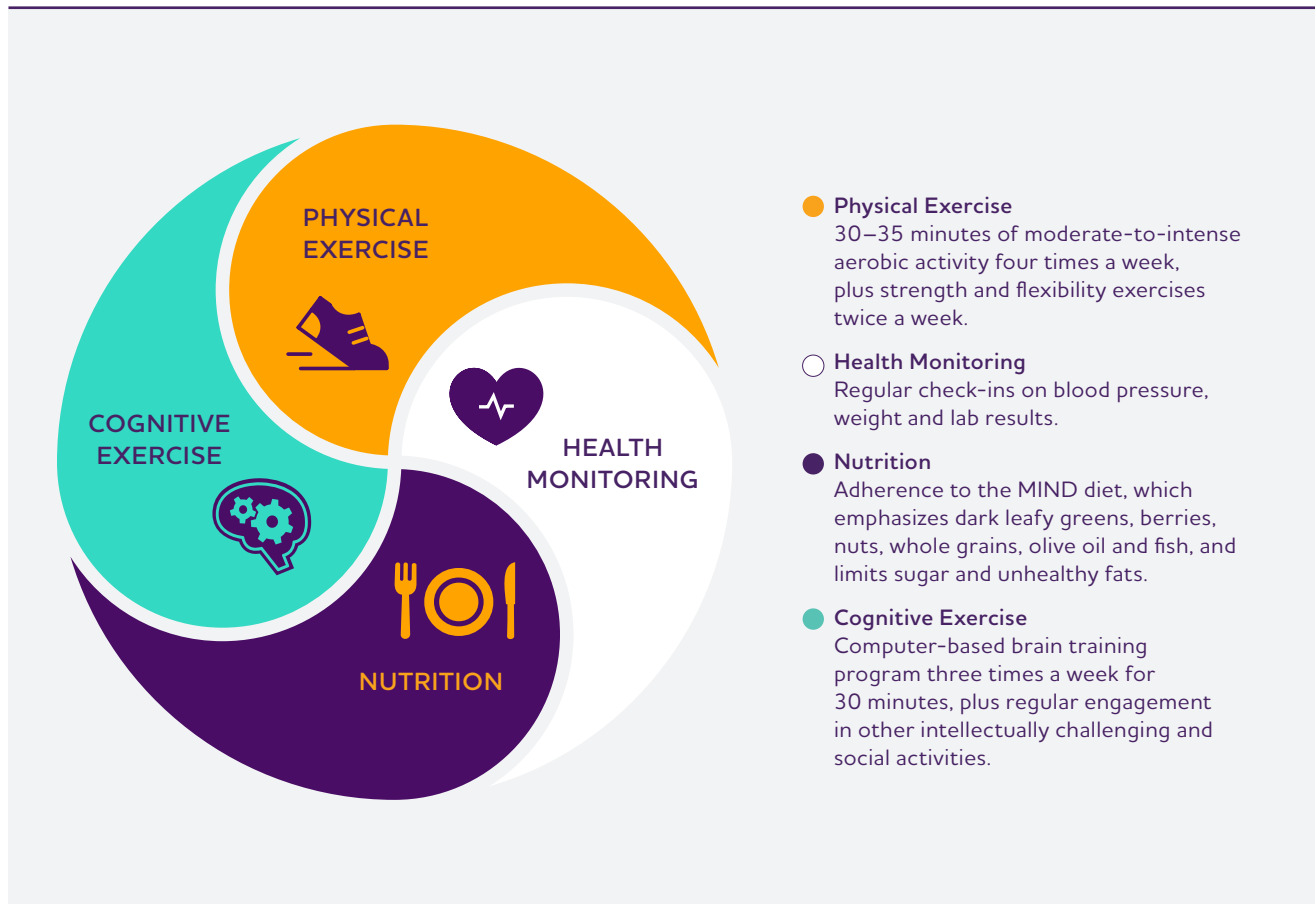
Many health conditions that influence cognitive function later in life, such as obesity, high blood pressure, diabetes and changes in sleep, first appear or become more common during midlife (see Potentially Modifiable Risk Factors section starting on page 19).^{87, 105, 107-112, 127-130}

This period is also one in which many individuals begin paying closer attention to their overall physical health, making midlife an important window for establishing habits that sustain brain health.

Daily behaviors typically associated with physical health — for example, regular movement, restorative sleep and nutritious eating — also play a central role in maintaining cognitive function and delaying cognitive decline. Modifiable cardiovascular-related risk factors are

FIGURE 20

U.S. POINTER Brain Health Recipe*



* U.S. POINTER = U.S. Study to Protect Brain Health Through Lifestyle Intervention to Reduce Risk. U.S. POINTER participants followed either a self-guided or structured lifestyle program. The two interventions both focused on the same lifestyle domains, but differed in structure, accountability and support provided. MIND = Mediterranean-DASH Intervention for Neurodegenerative Delay.

Created from data from Baker et al.²⁶⁸

especially important during this stage of life, and research shows a clear connection between mid-to-late-life heart health and the risk of cognitive decline.¹²¹

The concept of cognitive reserve also becomes particularly relevant during midlife, when its long-term implications are more apparent. Cognitive reserve refers to the brain's ability — developed through formal education, having a mentally stimulating job and engaging in mentally stimulating activities — to make flexible and efficient use of cognitive networks (networks of neuron-to-neuron connections) to enable a person to continue to carry out cognitive tasks despite changes to the brain.^{179,180} Cognitive reserve works like a mental savings account: the more one adds to the account throughout life, including in their late 30s through their early 60s, the more they can withdraw later if the brain is affected by cognitive-related diseases.

The goals and opportunities for supporting brain health may also differ between midlife and older adulthood. While later life has often emphasized protecting cognitive function and detecting early signs of decline, midlife offers a crucial opportunity to build long-term resilience. By adopting healthy lifestyle behaviors during this period, individuals could make lasting investments in their brain health.

What Americans Know About Brain Health and Brain-Healthy Lifestyle Choices

To better assess public perceptions of brain health — and the relationship between physical health, lifestyle behaviors and cognitive function — the Alzheimer's Association commissioned the University of Michigan (U-M) National Poll on Healthy Aging. The poll, directed by the U-M

Institute for Healthcare Policy & Innovation, surveyed a nationally representative sample of U.S. adults age 40 and older. The survey examined how people think about brain health, which behaviors they believe contribute to it, their current engagement in brain-healthy habits, and their preferences for receiving information and tools to support long-term cognitive health.

Key Findings

Perceptions of brain health

Americans age 40 and older show strong interest in brain health but lack clear guidance on how to support it.

- More than 4 in 5 adults (82%) associated “good brain health” with being able to think clearly and make good decisions, reflecting a functional, ability-based view of brain health.
- Nearly 9 in 10 (88%) said maintaining brain health as they age is very important, and 99% viewed brain health as equally or more important than physical health.
- Yet despite its perceived importance, fewer than 1 in 10 (9%) reported knowing “a lot” about ways to maintain brain health — signaling a significant knowledge gap.

Importance of brain health behaviors

Lifestyle behaviors are seen as critical yet not fully linked to dementia risk reduction.

- Three in 4 Americans (75%) believed lifestyle behaviors (e.g. diet, physical activity, sleep and mentally stimulating activities) play a very important role in maintaining brain health as they age.
- However, fewer than half of adults age 40 and older (46%) strongly associate lifestyle behaviors with reducing the risk of Alzheimer’s disease or other dementias.
- Adults rated other key behaviors as “very important” for supporting brain health as they age, including:
 - Protecting against head injuries (82%).
 - Not smoking (72%).
 - Managing stress (71%).
 - Treating depression (71%).

Engagement in brain health behaviors

While Americans age 40 and older overwhelmingly believe lifestyle behaviors support brain health, many do not practice them consistently.

- Despite strong beliefs in the importance of lifestyle behaviors, engagement is inconsistent.

- Sleep: Only half of adults (50%) get at least 7 hours of sleep daily/most days; 31% only one to three times per week.
- Diet: About 2 in 5 (39%) follow a healthy diet daily/most days; 40% only one to three times per week.
- Mental stimulation: About 2 in 5 (42%) perform some sort of activity daily/most days; 27% only one to three times per week.
- Exercise: 1 in 3 (34%) are physically active daily/most days; 36% only one to three times per week.
- About 1 in 3 survey participants said they engaged in mentally stimulating activities (31%) or physical exercise (30%) less than once a week or never.
- For adults who do engage in these habits at least weekly, maintaining brain health is a major motivator:
 - 83% said brain health motivates them to get enough sleep and do mentally stimulating activities.
 - 80% said it motivates their physical activity.
 - 79% said it motivates their healthy eating.

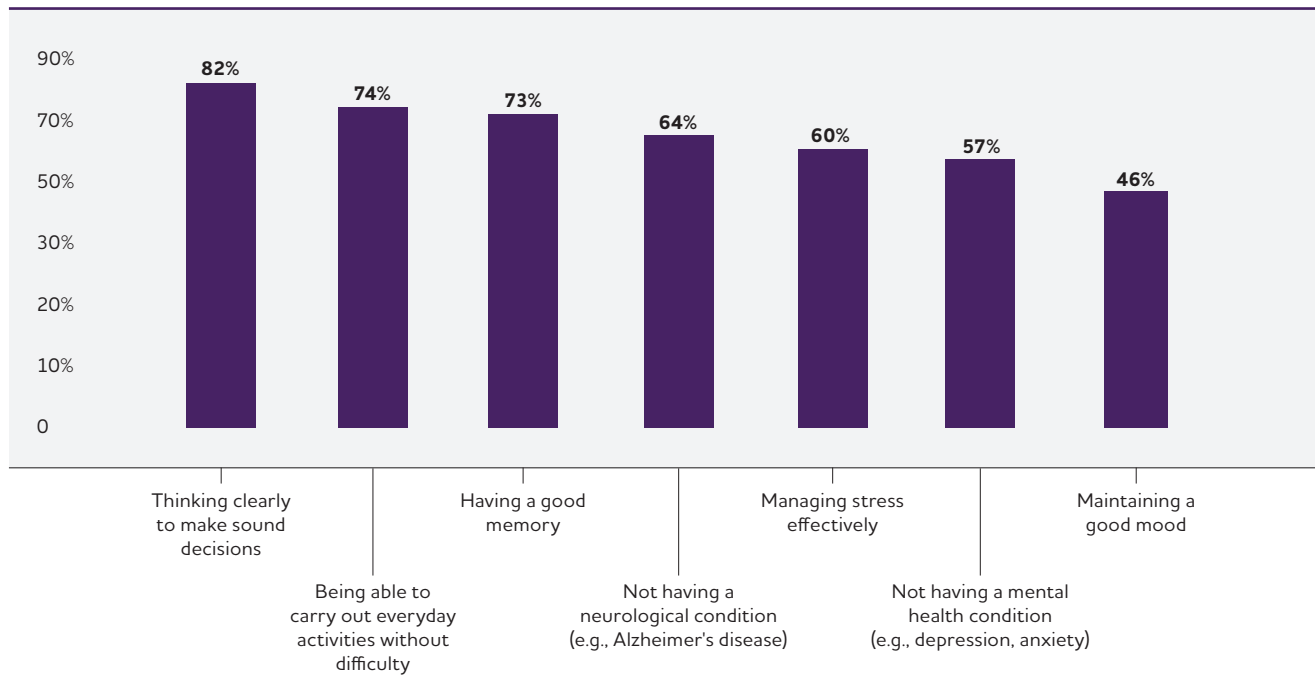
U.S. POINTER study and brain health lifestyle programs

There is high interest in brain health programs, but cost and logistics are major barriers.

- Awareness of U.S. POINTER, a major lifestyle-based brain health intervention research study, was low, with only 11% of Americans age 40 and older reporting familiarity.
- However, interest in brain health programs modeled after U.S. POINTER was strong, with nearly 3 in 4 Americans (73%) saying they were somewhat or very interested in participating.
- Adults preferred flexible formats that give them control over their effort:
 - 40% prefer self-guided activities at home.
 - 39% want a hybrid of self-guided and in-person sessions.
 - 8% prefer fully in-person programs with coaches and peers.
- Americans also rated how likely they were to engage in key U.S. POINTER program components, with cognitive exercise being the highest:
 - Cognitive exercise (57%).
 - Health monitoring (46%).
 - Nutrition (36%).
 - Physical exercise (26%).
- Cost was the most influential factor driving decisions to participate in a brain health lifestyle program (73%), followed by program location (67%), personal motivation (59%) and insurance coverage (58%).

FIGURE 21

What Good Brain Health Means to Adults Age 40 and Older



Importance of Midlife Engagement

Midlife may be an opportune time to act for better brain health.

- Many adults identified midlife (ages 35–64) as the critical window for action:
 - Nearly 2 in 5 (38%) said people should begin taking steps to support their brain health between ages 35–64.
 - Almost 1 in 2 (46%) said participation in a formal program should begin during this same period.
- About 1 in 3 Americans said doing things to support brain health should be a lifelong endeavor, with action appropriate at any age.

Interest in resources and tools for brain health

Americans age 40 and older want guidance on brain health from trusted health care professionals, but these conversations rarely occur.

- Two in 3 Americans (66%) said they prefer to learn about brain health from their health care provider, and most individuals (86%) want brain health information during routine care.
- Yet only 14% of adults report having a conversation about maintaining brain health with their physician; even fewer (11%) have discussed ways to reduce the risk of Alzheimer's and other dementias.

Survey Results

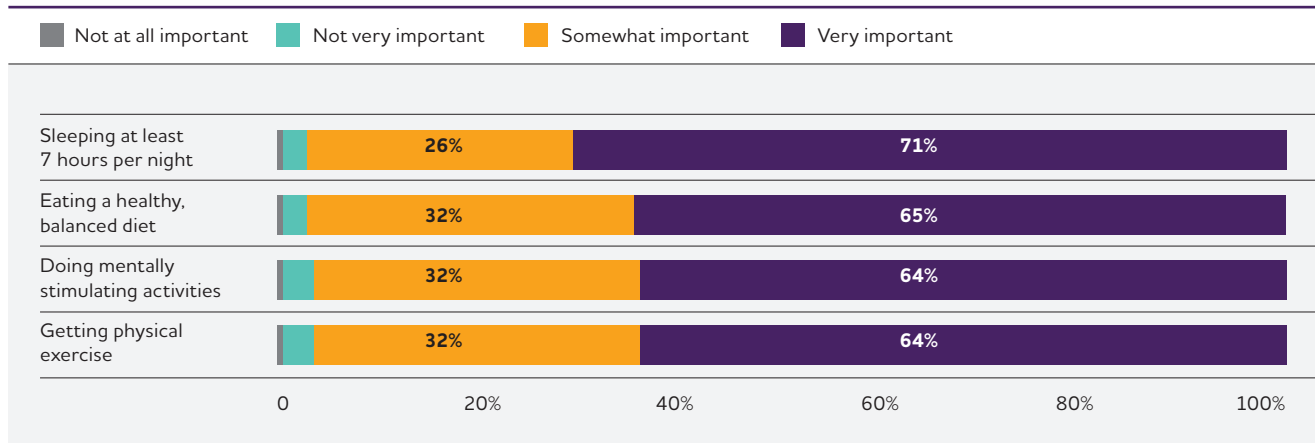
A survey of 3,829 U.S. adults age 40 and older was conducted from December 29, 2025, to January 13, 2026. The sample was sourced via NORC's AmeriSpeak Panel at the University of Chicago. AmeriSpeak is a probability-based panel of all U.S. households. The full sample included oversampling of adults identifying as American Indian or Alaska Native (n=162), Asian (n=298), Black (n=495) and Hispanic (n=485) for robust analysis of subgroups. The final data were weighted by select criteria to match population totals from the U.S. Census Bureau. The survey was offered in English as an online or phone survey.

Americans age 40 and older primarily view good brain health as functional ability rather than the absence of disease or mental well-being

Survey participants overwhelmingly defined brain health in terms of what they can do, emphasizing cognitive ability and day-to-day functioning rather than emotional well-being or the absence of disease. Survey participants most often associated good brain health with thinking clearly to make sound decisions (82%), carrying out everyday activities without difficulty (74%) and having a good memory (73%) (Figure 21). These findings suggest that Americans view good brain health as fundamental to self-reliance, autonomy and control over their lives as they age. Functional abilities were rated higher than not having

FIGURE 22

Perceived Importance of Health Behaviors in Maintaining Brain Health



a neurological condition (64%) or a mental health condition (57%). Similarly, cognitive performance outweighed responses related to emotional and psychological well-being. While 60% cited managing stress as part of good brain health and 57% mentioned not having a mental health condition such as depression or anxiety, fewer respondents associated brain health with maintaining a good mood, which was rated lowest at 46%.

Americans have high confidence in their own brain health, although there is minimal understanding of how to support it

When asked to self-rate their current brain health, responses skewed heavily positive, with the majority (90%) reporting good to excellent brain health. More than 1 in 2 adults (55%) said it was excellent (13%) or very good (42%), and another 35% considered their brain health to be good. Only 1 in 10 adults viewed their brain health as suboptimal (fair, 9% or poor, 1%).

However, this high baseline confidence in the state of their own brain health contrasted sharply with participation in activities that are important to brain health or understanding of how to actively preserve cognitive function. Very few survey participants (9%) claimed to know a lot about ways to maintain brain health. The vast majority admitted to knowing only some (51%), a little (31%) or nothing (8%).

Brain health is viewed as universally important and on par with physical health

When asked about the overall value of keeping their cognitive abilities strong as they age, nearly all adults 40 and older (99%) reported that it is important, with most reporting it was very important (88%). Furthermore,

respondents viewed brain health as being just as critical as physical health, if not more so. When asked to compare, nearly 9 in 10 adults (87%) said maintaining brain health is equally important as maintaining physical health, 12% considered brain health to be more important, and only 1% placed physical health above brain health.

Americans age 40 and older overwhelmingly believe lifestyle behaviors support brain health, but many do not practice them consistently

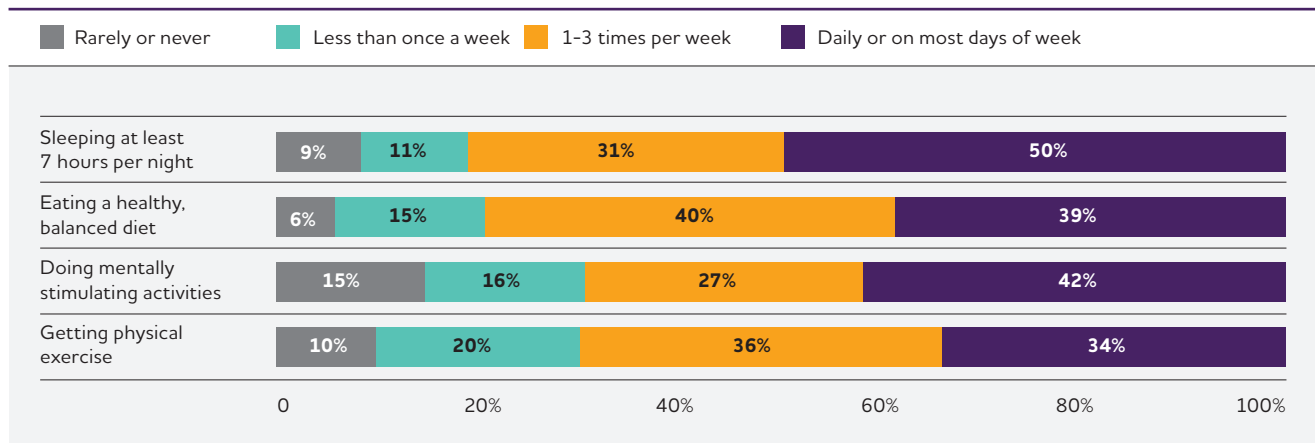
Americans overwhelmingly believe that everyday lifestyle behaviors — such as good sleep, healthy diets, physical activity and mentally stimulating activities — are important for preserving brain health. Nearly all adults surveyed (99%) said these habits matter for overall cognitive well-being, with 3 in 4 (75%) rating them as very important and the remaining 24% viewing them as somewhat important.

Sleep emerged as the most valued lifestyle behavior, with nearly 3 in 4 (71%) saying it was very important to maintain brain health (Figure 22). Respondents also placed strong importance on eating a healthy, balanced diet (65%), doing mentally stimulating activities (64%) and getting physical exercise (64%).

Despite these strongly held beliefs, engagement in these behaviors was inconsistent. Sleep was the most consistent habit, but even then, only half of adults (50%) reported getting at least 7 hours of sleep daily or on most days, with another 31% doing so only one to three times a week (Figure 23).

FIGURE 23

Engagement in Brain Health Behaviors in the Past Year



Participation in other healthy behaviors was even less consistent:

- Eating a healthy diet: 39% daily/most days; 40% one to three times a week.
- Doing mentally stimulating activities: 42% daily/most days; 27% one to three times a week.
- Getting physical exercise: 34% daily/most days; 36% one to three times a week.

About 1 in 3 adults said they rarely or only occasionally engage in mentally stimulating activities or physical exercise. For mental stimulation, 15% report doing it rarely or never and 16% less than once a week. Similarly, 10% of adults reported rarely or never exercising, and 20% less than once a week (Figure 23).

For adults who practiced these behaviors at least weekly, keeping their brain healthy was a strong motivator. Four in 5 adults (83%) said maintaining brain health motivates them to get enough sleep and to do mentally stimulating activities, with 45% in both categories saying it motivated them a lot. Brain health was also cited as a significant motivator for those getting regular physical exercise (80%, some or a lot) and eating a balanced diet (79%, some or a lot).

When presented with a broader list of modifiable health factors, survey participants prioritized physical safety and mental health (Table 25). Protecting against head injuries was the highest-rated factor overall, with 4 in 5 adults (82%) rating it as very important for maintaining brain health. Avoiding harmful habits and addressing mental well-being were also seen as crucial, with not smoking (72%), managing stress (71%) and treating depression (71%) rounding out the top strategies.

Americans age 40 and older worry about Alzheimer's disease and dementia, but are slightly less convinced of the role of healthy lifestyle behaviors in lowering their risk

Worry about Alzheimer's disease and other dementia is widespread, yet many adults remain uncertain about whether adopting healthy lifestyle behaviors can reduce their risk of dementia. Among adults age 40 and older, 68% said they were worried about developing Alzheimer's disease or other dementia, with 1 in 5 (20%) classifying themselves as very worried and 48% as somewhat worried. Only 1 in 3 (32%) report not being worried. Despite this concern, confidence in the ability of everyday habits to lower their risk of Alzheimer's or other dementia is more modest. While nearly all adults (93%) believe lifestyle behaviors are at least somewhat important in reducing dementia risk, fewer than half (46%) viewed habits such as sleep, diet and physical activity as very important. Nearly half (47%) consider them only somewhat important and 8% said they were not important. These findings suggest that many Americans have limited awareness of the scientific evidence demonstrating that individuals who address modifiable risk factors may help decrease their risk of Alzheimer's disease or another type of dementia.

There is high interest in brain health programs despite limited research awareness, with cost a primary driver of program participation

Despite low awareness of current clinical research on lifestyle programs and brain health, there is robust enthusiasm for such programs among Americans age 40 and older. Survey participants were largely unfamiliar with the U.S. POINTER study (88% not very or at all familiar). More than half (57%) also reported low familiarity with lifestyle and brain health research more broadly. Despite

this low awareness and uncertainty about the impact of lifestyle behaviors on dementia risk as discussed previously, nearly 3 in 4 adults (73%) were somewhat or very interested in participating in a brain health program modeled after U.S. POINTER (24% very; 49% somewhat; Figure 24). For a reminder of the U.S. POINTER components, refer to Figure 20.

However, translating this broad interest into actual participation hinges heavily on practical logistics. For 3 in 4 adults (73%) financial cost was cited as the most significant consideration to joining a program. Other deciding factors of participation included (marked very important; Figure 25):

- Location of program (67%).
- Personal motivation (59%).
- Insurance coverage (58%).
- Coverage through employer (51%).
- Length of program (49%).
- Recommendation from health care provider (31%).
- Recommendation from family or friends (15%).

Figure 25 shows the full range of responses and how survey participants rated each factor. Overall, the findings suggest that while adults are highly receptive to structured brain health programs, widespread participation will require addressing cost and logistical barriers. Traditional social influences (recommendations from physicians or family) carried less weight.

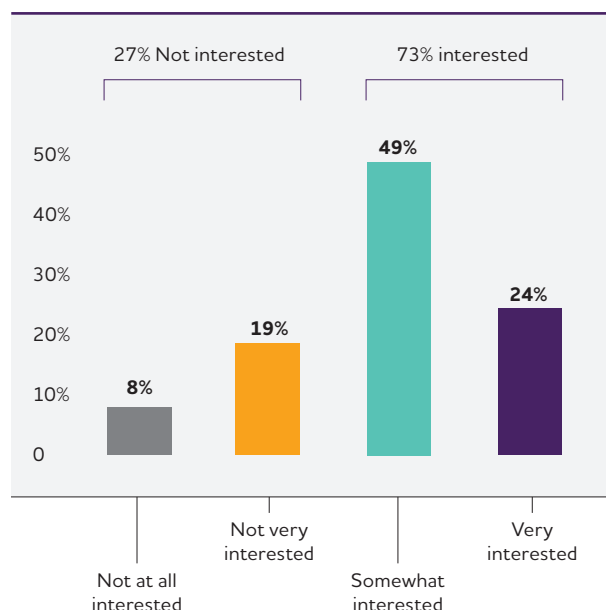
TABLE 25

Perceived Importance of Other Factors in Maintaining Brain Health

Factors	Rated as Very Important
Protecting against head injuries	82%
Not smoking	72%
Managing stress	71%
Treating depression	71%
Controlling blood pressure	67%
Avoiding falls	65%
Maintaining a healthy weight	57%
Socializing with others	55%
Limiting alcohol	55%
Addressing hearing loss	54%
Managing cholesterol	51%

FIGURE 24

Interest in Participating in a Brain Health Program



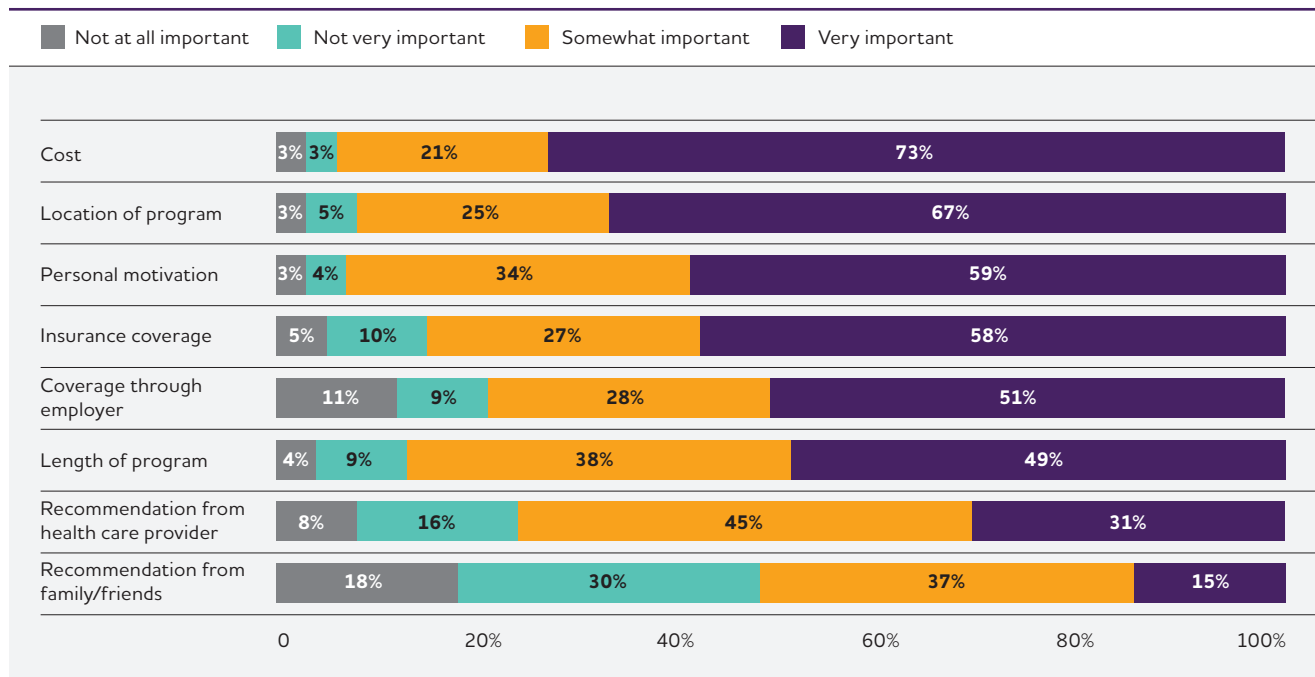
Midlife is viewed as the prime time to begin addressing cognitive health through flexible, self-guided brain health programs

More than 1 in 3 adults (38%) believe people should start taking active, self-guided steps to support their brain health in the 35-to-64 midlife window, and 46% said participation in a formal brain health program should begin during this time (Figure 26). Nearly 1 in 4 respondents (24%) felt individuals should start taking everyday steps to support brain health before age 35. Adults age 40 to 64 were significantly more likely than those 65 and older to say people should begin taking steps to mitigate cognitive decline before age 50. Approximately 1 in 3 adults viewed brain health as a lifelong endeavor, stating that individuals should take personal action (29%) and participate in programs (26%) at any age.

When asked what they wanted to gain from participating in brain health lifestyle activities, participants prioritized decreasing the likelihood of disease and enhancing cognitive function. Approximately 3 in 4 survey participants cited lowering the risk of Alzheimer's disease or other dementia (74%) and achieving better memory and thinking skills (72%) as top goals. Many were also motivated by broader health improvements, including:

FIGURE 25

Importance of Factors in Deciding to Participate in a Brain Health Lifestyle Program



- Improved physical fitness and strength: 60%.
- Better control of health factors: 55%.
- Better sleep quality: 53%.
- Reduced stress: 48%.
- Healthier eating habits: 47%.
- Stronger social connections: 22%.

When it comes to specific activities that might be part of a brain health program, respondents were most receptive to workouts for the mind. Over half (57%) said they would be very likely to engage in cognitive exercises, and 46% were very likely to engage in health monitoring. In contrast, fewer adults said they would be very likely to participate in the nutrition (36%) and physical exercise (26%) components of a structured program, suggesting many adults age 40 and older still characterize brain health as a cognitive activity, rather than something that is addressed holistically through attention to physical health as well.

Finally, adults strongly prefer flexibility in how these programs are delivered, with 4 in 5 (79%) favoring either fully self-guided activities at home (40%) or a combination of in-person and self-guided activities (39%). Only 8% preferred fully in-person programs. This strong preference for flexible, self-guided formats suggests that many adults want brain health programs they can adapt to their own circumstances, an approach that may be especially appealing during midlife.

Americans age 40 and older want guidance on brain health from trusted health care professionals, but these conversations rarely occur

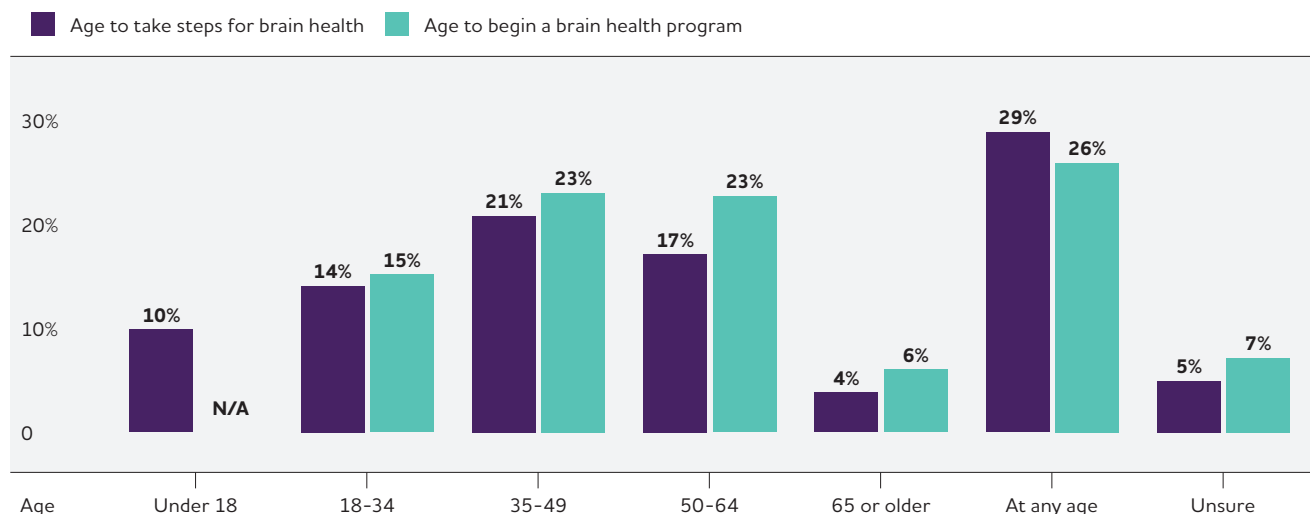
When asked about how they'd like to learn about brain health, 2 in 3 respondents (66%) identified their health care provider as their top source for information (Figure 27). Although there was strong trust in health care providers as the primary source for brain health information, only 14% of adults have discussed this topic with their provider. This may be a reflection of limited public awareness and the absence of established clinical guidance on brain health. It also spotlights a major opportunity to integrate brain health into routine care as guidance and tools become more available.

Respondents also expressed a clear desire for health care providers to initiate brain health conversations proactively. Nearly 9 in 10 (86%) said they want brain health education during an annual exam, regardless of whether they are experiencing memory issues. Many also supported other proactive approaches, including:

- After a cognitive screening shows memory and thinking problems: 41%.
- A health care visit devoted just to discussing brain health: 36%.
- An information session offered by a health system/health care provider: 35%.
- Communication from a patient portal: 33%.
- After being diagnosed with dementia: 19%.

FIGURE 26

Suggested Age at Which People Should Take Steps to Support Brain Health or Participate in a Brain Health Lifestyle Program



Only 2% of those surveyed were not interested in brain health information from their health care provider.

Alongside clinical guidance, respondents showed strong interest in self-directed learning, including articles or other written materials (49%), online webinars or videos (33%), and apps on a smartphone or tablet (29%) (Figure 27).

Employed adults show openness to brain health education through workplace offerings

Workplaces also emerged as a possible channel for reaching adults with brain health information. Among employed respondents, 18% said they would be interested in receiving brain health information through their workplace or employee wellness programs (Figure 27). Overall, practical, accessible formats were favored. Among adults interested in workplace programs, more than 3 in 4 (77%) adults wanted lifestyle tips and tools, 63% were interested in one-on-one brain health coaching and 61% desired referrals to health clinics. Group activities focused on brain health (52%) and guest speakers or expert talks (40%) were also popular options for workplace integration.

Age, gender, race and ethnicity, family history of dementia, education and household income influenced some responses

Adults shared different perspectives and experiences depending on their age, gender, race and ethnicity, family history of Alzheimer's disease or dementia, education and household income.

Age

Adults age 40 to 64 expressed greater worry about developing Alzheimer's disease than older Americans (23% vs. 15% said they were "very worried"). More than 2 in 3 adults 65 and older (67%) said they know some or a lot about ways to support brain health compared with 56% of adults 40 to 64. Adults age 65 and older also reported more regular engagement in all healthy behaviors. In addition, adults 40 to 64 showed stronger interest in a brain health program (28% vs. 20%) and were more inclined to say such programs should begin in midlife than Americans 65 and older.

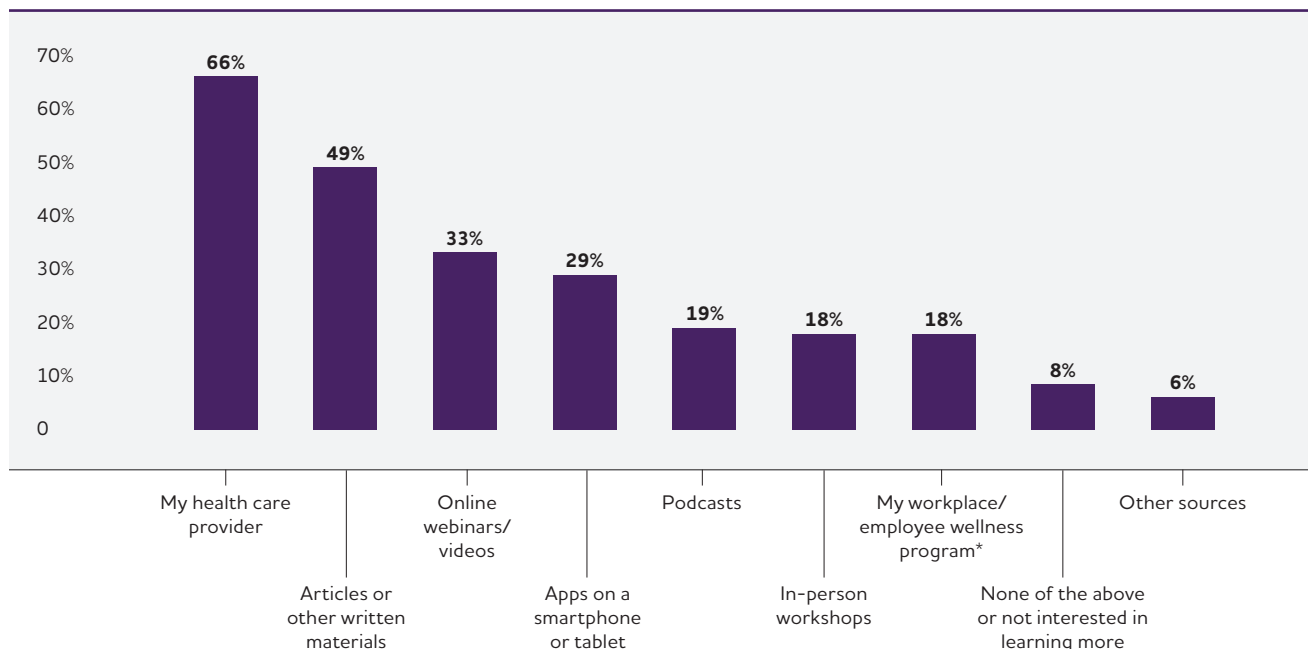
Gender

Women expressed greater concern about developing Alzheimer's than men (23% vs. 16% said they were "very worried"), and were more likely to say they knew some or a lot about ways to support brain health (65% vs. 56%). Women also reported slightly more interest in participating in brain health programs than men (76% vs. 70%).

Women consistently rated lifestyle behaviors as very important at higher levels than men and reported that maintaining brain health motivated them a lot to eat a healthy and balanced diet (40% vs. 30%) and do mentally stimulating activity at least weekly (51% vs. 39%).

FIGURE 27

Sources of Information About Brain Health



*This item was shown only to those who indicated they were employed (n=1,908).

Race and ethnicity

Interest in a brain health lifestyle program was especially strong among American Indian or Alaska Native, Black, and Hispanic adults (31–39%) compared with white adults (21%). Black and Hispanic adults also placed higher importance on lifestyle behaviors for reducing the risk of Alzheimer's and related dementias. For example, 60% of Hispanic adults rated lifestyle as "very important," compared with 40% of white adults.

Family history

Adults with a family history of Alzheimer's expressed substantially higher worry about developing the disease (80% vs. 57%) than those without a family history and were slightly more likely to have discussed brain health and dementia risk reduction with a health care provider. Despite these differences, their interest in brain health lifestyle programs was no greater than that of those without a family history.

Education

Adults with a high school education or less were more likely to be very worried that they will develop Alzheimer's disease than those with a bachelor's degree or higher (23% vs. 18%).

Education was also linked to healthier lifestyle patterns. Adults with at least a bachelor's degree were more likely to

rate lifestyle behaviors as very important for maintaining brain health. They were also more likely than those with less education to report regular:

- Physical exercise (41 vs. 28%).
- Mentally stimulating activities (50% vs. 35%).
- Healthy diet (48 vs. 29%).

Higher education was further associated with greater interest in participating in a brain health program (31% very interested among those with a bachelor's degree vs. 23% some college and 19% high school or less).

Household income

Adults with lower household incomes expressed the greatest worry about developing Alzheimer's disease or another dementia. Among adults 40 and older with annual incomes below \$30,000, a substantially higher share reported being very worried compared with those with higher household incomes (28% vs. 17%).

Income was also reflected in differing barriers to participating in brain health programs. Adults with lower household incomes were more likely to consider cost and cite physical or health limitations, technology challenges and transportation as barriers, whereas adults with higher incomes were more likely to cite time constraints as the primary barrier.

Building a Coordinated Framework to Advance Brain Health Across the Lifespan

This year's Special Report reveals that Americans value brain health, feel motivated to protect it and want guidance they can trust but face gaps in knowledge, access and system-level frameworks to do so. These insights reinforce the need for coordinated, research-backed programming that meets individuals where they are and helps them take meaningful action to support brain health across the lifespan. Together, these findings show that adults want accessible, trustworthy information about maintaining brain health, ideally delivered through both health care settings and self-directed formats.

The Alzheimer's Association is working with stakeholders across multiple sectors to create a future that emphasizes brain health across the lifespan to ensure individuals, communities, workplaces and health care systems receive consistent, evidence-based guidance. While supporting individuals with Alzheimer's disease and their families remains a priority, the Alzheimer's Association is significantly increasing its focus on reaching and helping people earlier, sometimes long before symptoms begin, to promote behavior changes that support lifelong brain health. Programs and initiatives informed by the U.S. POINTER study are offered at a variety of touchpoints. This approach moves beyond individual decision-making to shape the environments where people live, work and receive care.

Brain Health for Individuals

Most brain health-focused tools and programming available today are self-guided, a format that aligns closely with survey findings showing that adults prefer autonomy, flexibility and resources they can integrate into their daily routines. The Alzheimer's Association is spearheading efforts to make brain health information accessible to more Americans. This includes nationwide education and tools such as the *10 Healthy Habits for Your Brain* campaign, ALZ Talks webinars covering topics ranging from brain health to dementia to caregiving, and the Brain Health Habit Builder. The Brain Health Habit Builder is a free, web-based tool that offers a quick, interactive experience for people to assess their brain health habits and develop a tailored action plan with research-backed guidance, including learnings from the U.S. POINTER study.

These tools help address the gaps between confidence and knowledge that the survey revealed by translating scientific evidence into simple, actionable steps. They also reflect adults' desire for personalized guidance: the Brain Health Habit Builder provides tailored action plans encouraging small, meaningful changes in areas like physical activity, diet, sleep and cognitive engagement.

Brain Health in Communities

The survey revealed that adults view a healthy brain as essential to independence and well-being as they age, yet many lack an understanding of evidence-based strategies that can help them achieve it. Many respondents also expressed strong motivation to take action on their own, particularly adults in midlife and underrepresented racial and ethnic groups, but reported uncertainty about where to start. These findings point to a clear opportunity: community-based programs can meet people where they live, work and learn to give practical, trustworthy information. This serves to normalize conversations and increase visibility of brain health programs that reinforce personal agency and provide concrete steps toward brain-healthy habits.

The Alzheimer's Association's Building Brain-Healthy Habits program is a free, volunteer-led program that teaches science-backed strategies for incorporating brain-healthy habits into daily life. During presentations, participants are guided in creating a personalized six-step action plan that includes goal-setting and strategies for anticipating and overcoming potential barriers to lifestyle change. The program is designed for use across a wide range of community and workplace settings, with a flexible format that allows it to be delivered in libraries, community centers, gyms, coworking spaces, corporate wellness programs, universities, adult learning centers, faith-based settings and other local gathering places as well as online.

Brain Health in the Workplace

The workplace is one of the most powerful settings to support lifelong brain health, especially for reaching people in midlife. This year's Special Report survey found that many employed adults are open to receiving practical tools, coaching and referrals about brain health through their workplaces, presenting an opportunity to reach working-age adults, including those who may be caring for someone living with dementia or be at elevated risk of developing cognitive issues themselves.

From the employer perspective, brain health is one component of the growing concept of "brain capital," which contributes to workforce capacity, economic growth and national competitiveness. Brain capital includes brain health and brain skills.¹⁰⁴⁷ Brain skills encompass the cognitive, interpersonal, self-management and technological abilities that draw on higher-order thinking, emotional regulation and adaptive capacity. Promoting brain health can help foster these skills as individuals age.¹⁰⁴⁷

The Alzheimer's Association is leading a national movement to make brain health a workplace priority. The recently launched Brain Health At Work initiative is a no-cost program designed to help employers create brain-healthy workplaces, reduce employees' risk of cognitive decline and

support working caregivers. Through Brain Health At Work, employers receive immediate access to evidence-informed tools that support brain-healthy habits — such as staying active, eating well, challenging the mind and managing chronic conditions — as well as resources to support working dementia caregivers. Through the initiative's Workplace Recognition Program, companies can also view dashboard results, compare their progress with peer organizations and earn recognition through digital badges and certificates that signal their commitment to employee well-being and caregiver support.

By pairing programming that supports cognitive functioning with employer-driven efforts to strengthen brain skills, organizations can reinforce how essential brain health is for individual quality of life and a thriving, adaptable workforce.

Brain Health in Health Care Settings

Survey findings reveal a disconnect between what adults age 40 and older want regarding brain health during a doctor's visit and what actually occurs. They overwhelmingly trust their health care providers for brain health information, yet few have ever discussed it. Together, these findings point to a gap between patient expectations and current clinical practice around brain health and lifestyle guidance.

Developing clinical practice guidelines for brain health

There is an increasing need for clear, actionable recommendations to support brain health. Although nearly all adults 40 and older view brain health as as important as physical health and identify health care providers as a trusted source of information, relatively few report receiving counseling or having discussions about brain health during routine clinical visits. To help address this gap, the Alzheimer's Association is collaborating with multidisciplinary experts, health systems and professional societies to develop consensus-driven clinical practice guidelines for brain health across the lifespan. These efforts build on the Association's longstanding commitment to evidence-based guidance and are intended to support earlier, more consistent and more equitable integration of brain health into routine care.

Building clinical expertise in brain health and dementia care

The Alzheimer's Association is positioned to help close the current clinical gap through new interprofessional continuing education offerings, including live conferences, virtual programs, online training and quality-improvement-focused education. With recently earned Joint Accreditation status, the Association can train entire care teams through a single, unified national credential.¹⁰⁴⁸ This training is designed for physicians, nurses, social workers, pharmacists and others. These offerings will help health

systems, health plans and community partners build workforce capacity in brain health for people of all ages, as well as aid in early detection, diagnosis, care coordination and treatment navigation for individuals at risk of or diagnosed with dementia.

The Association also supports health care professional development in dementia care through the Alzheimer's and Dementia Care ECHO Program. This program follows the Project ECHO® telementoring model, connecting primary care teams with dementia specialists to build skills in risk reduction, diagnosis and care management. The latest ECHO Series, Alzheimer's and Dementia Care for Brain Health, is designed to support primary care teams in promoting brain health across the lifespan. The series focuses on practical, evidence-based approaches that can be integrated into routine primary care workflows. Through regular case-based learning, ECHO programs help clinicians gain confidence, improve care quality and expand access to expertise in communities where specialty care is limited.

Integrating healthy brain principles into routine care

Training provides the foundation, but clinicians also need practical methods to integrate brain health into everyday workflows. The Alzheimer's Association is developing resources to help clinicians initiate conversations and incorporate brain health into practice.

Meaningful behavior change often requires more than passive discussion. Evidence from U.S. POINTER indicates that individuals who receive greater structure and support for lifestyle modification experience greater cognitive benefits. Care teams could take a more active approach by:

- Sharing essential knowledge, such as evidence-backed recommendations to help individuals form or sustain brain-healthy habits.
- Explaining the importance of a structured lifestyle "prescription" to reach brain health goals.
- Referring patients to community and institutional resources that can provide ongoing assistance.

The recent evidence from U.S. POINTER strengthens the case for including brain health as part of regular health care visits. Clinicians could begin by:

- Initiating brief brain health discussions during cardiovascular and metabolic visits.
- Assessing a patient's readiness to make changes and helping them establish accountability for adopting or continuing healthy behaviors.
- Connecting patients to community programs, wellness coaches, dietitians, exercise specialists, faith-based groups or even family and friends who can help promote brain-healthy behaviors.

Establishing a comprehensive resource hub for health care professionals

ALZPro® is the Alzheimer's Association's central hub for professional resources on brain health and dementia. It brings together care resources, research, data platforms, clinical guidance and education in one place to support real-world use. Designed for a wide range of professionals, including clinicians, researchers, health systems and public health leaders, ALZPro helps advance early detection, improve care and expand equitable access. Users can access evidence-informed resources, tools, and insights to support decision making, stay current with emerging science and apply best practices in their work.

Brain Health Across Populations

In addition to the individual- and setting-specific efforts described in earlier sections, the Alzheimer's Association is contributing to the public health dialogue needed to advance brain health at the population level.

Many lifestyle behaviors highlighted previously do not happen in isolation. Community environments can either facilitate or hinder healthy behaviors, making public health strategies essential for supporting brain health across the lifespan. These strategies often focus on improving the social and environmental conditions that shape people's daily lives. For example, communities can promote healthier eating by introducing farmers markets to underserved neighborhoods, establishing strong nutrition standards in childcare settings and schools, and supporting community gardens and urban agriculture.

Economic stability, another key driver of health, can be strengthened through improved access to affordable health care, summer job programs and workforce training for youth, financial literacy initiatives and incentives that help small businesses grow in economically underserved areas. In addition, investments in healthier physical environments — such as planting more trees, expanding green spaces and walkable neighborhoods, promoting renewable energy and improving public transportation and bike path networks — can help foster active lifestyles, reduce pollution and support long-term brain health.

To help communities put these strategies into practice, the Alzheimer's Association Public Health Center of Excellence on Dementia Risk Reduction, funded by the Centers for Disease Control and Prevention (CDC), translates the latest science on dementia risk reduction into actionable tools, materials and messaging. The resources enable public health agencies to promote brain health and reduce dementia risk among all populations, including those in diverse, underserved and higher-risk communities.

Another example of activating public health strategies in local communities is the Healthy Brain Initiative (HBI) Road Map. For nearly 20 years, the HBI Road Map — a joint project of the CDC and the Alzheimer's Association — has provided public health professionals a framework to promote brain health across the lifespan and support dementia caregivers. In 2019, the Healthy Brain Initiative Road Map for American Indian and Alaska Native Peoples became the first guide tailored to the needs of these communities; it was updated in 2024. Since 2022, the Alzheimer's Association Healthy Brain Initiative has funded 67 local and tribal health organizations to put the HBI Road Map into practice, elevating brain health, dementia and caregiving as public priorities in these communities.

In light of the positive results of the U.S. POINTER clinical trial, there is a desire to see such programs established in communities across the country. The Alzheimer's Association is funding efforts on implementation science — the public health discipline focused on translating successful clinical trials to real-world practice in communities. In addition, a new initiative from the federal government, Make America Healthy Again: Enhancing Lifestyle and Evaluating Value-based Approaches Through Evidence (MAHA ELEVATE), will be providing funds to test innovative programs, such as U.S. POINTER, that promote health and prevention of chronic diseases among Medicare beneficiaries. The federal government could take a more aggressive approach at catalyzing community implementation of U.S. POINTER by specifically funding U.S. POINTER implementation studies through the CDC and the National Institutes of Health.

Together, these efforts position the public health community to improve brain health at the population level through coordinated, community-driven action.

This year's Special Report highlights a critical moment to move beyond viewing brain health as an issue of older age and instead establish it as a lifelong priority and a shared responsibility across society. While individual behaviors matter, lasting progress depends on coordinated actions across workplaces, communities, health care systems and public health institutions to optimize the conditions that support cognitive function and well-being. By connecting strategies at the individual, community and population levels, the nation can build a more equitable and accessible framework for brain health—one that empowers people of all ages and backgrounds to protect cognitive function and reduce dementia risk throughout their lives.

Appendices

End Notes

- A1. Racial and ethnic identifiers: *Facts and Figures* keeps the racial and ethnic terms used in source documents when describing study findings. When not referring to data from specific studies, adjectives such as “Black,” “Hispanic” and “White” may be used (for example, Black populations and Hispanic communities).
- A2. Estimated prevalence (number and proportion) of Americans age 65 and older with clinical Alzheimer’s dementia for 2026: The estimated 7.4 million individuals ages 65 years and older with clinical Alzheimer’s dementia and the estimated numbers of individuals with clinical Alzheimer’s dementia in each age group are from a study that used data from the Chicago Health and Aging Project (CHAP) in combination with population projections from the U.S. Census.²⁷⁶ The number 7.4 million for 2026 comes from a linear extrapolation of the reported projections for 2025 (7.16) and 2030 (8.53) in the CHAP study. The percentage of people with clinical Alzheimer’s dementia (among people age 65 and older and by age group) is calculated using the numbers from the CHAP study as the numerators and the U.S. Census population projections for the specific age groups of interest as the denominators.
- A3. Differences between CHAP and HRS-HCAP estimates for Alzheimer’s dementia prevalence: The number of people estimated to have any form of dementia in the U.S. in 2016 from the Health and Retirement Study’s (HRS) Harmonized Cognitive Assessment Protocol (HCAP: 4.92 million) is lower than the CHAP estimate of how many people were living with clinical Alzheimer’s dementia only (6.07 million).¹⁷⁵ This is because of differences in dementia ascertainment between the two studies: both studies used scores on batteries of cognitive tests, but the HRS-HCAP study additionally required an informant report of functional impairment (i.e. disability). Because the more stringent threshold for dementia in HRS-HCAP may miss people with mild Alzheimer’s dementia, the Association believes that the larger CHAP estimates may be a more relevant estimate of the burden of clinical Alzheimer’s dementia in the United States.
- A4. Criteria for identifying people with Alzheimer’s or other dementias in the Framingham Heart Study: From 1975 to 2009, 7,901 people from the Framingham Study who had survived free of dementia to at least age 45, and 5,937 who had survived free of dementia until at least age 65 were followed for incidence of dementia.³²⁹ Diagnosis of dementia was made according to the Diagnostic and Statistical Manual of Mental Disorders, 4th Edition (DSM-IV) criteria and required that the participant survive for at least 6 months after onset of symptoms. Standard diagnostic criteria (the NINCDS-ADRDA criteria from 1984) were used to diagnose clinical Alzheimer’s dementia. The definition of Alzheimer’s and other dementias used in the Framingham Study was very strict; if a definition that included milder disease and disease of less than six months’ duration were used, lifetime risks of Alzheimer’s and other dementias would be higher than those estimated by this study.
- A5. Projected number of people with Alzheimer’s dementia, 2020–2060: This figure comes from the CHAP study.²⁷⁶ Other projections are somewhat lower (see, for example, Brookmeyer et al.¹⁰⁴⁹) because they relied on more conservative methods for counting people who currently have clinical Alzheimer’s dementia.⁴³ Nonetheless, these estimates are statistically consistent with each other, and all projections suggest substantial growth in the number of people with clinical Alzheimer’s dementia over the coming decades.
- A6. Annual mortality rate due to Alzheimer’s disease by state: Unadjusted death rates are presented rather than age-adjusted death rates in order to provide a clearer depiction of the burden of mortality for each state. States such as Florida with larger populations of older people will have a larger burden of mortality due to Alzheimer’s — a burden that appears smaller relative to other states when the rates are adjusted for age.
- A7. Number of family and other unpaid caregivers of people with Alzheimer’s or other dementias: To calculate this number, the Alzheimer’s Association started with data from the Behavioral Risk Factor Surveillance System (BRFSS) survey. Since 2016, all states and the District of Columbia utilized the BRFSS caregiver module. This module identified respondents age 18 and over who had provided any regular care or assistance during the past month to a family member or friend who had a health problem, long-term illness or disability. The module asks a series of follow-up questions, including asking the caregiver to identify what the main health problem, long-term illness, or disability that the person they care for has. One of the reported condition categories is “Alzheimer’s disease, dementia, or other cognitive impairment.” In the BRFSS surveys conducted in 2019 and after, an additional follow-up question was included, asking if the caregiving recipient also had dementia in addition to their main condition. Prior to 2019, the survey did not include caregivers of recipients for whom dementia was not their main condition, so these numbers were imputed using data collected in 2019 by the National Alliance for Caregiving (NAC)/AARP survey. The NAC/AARP survey asked respondents age 18 and over whether they were providing unpaid care for a relative or friend age 18 or older or had provided such care during the past 12 months. Respondents who answered affirmatively were then asked about the health problems of the person for whom they provided care: 11% of respondents reported dementia as the main condition of their care recipient, while 26% of all respondents reported the presence of dementia. Using this ratio in combination with BRFSS data, the Alzheimer’s Association was able to determine the percentage of adults in all states and the District of Columbia who are caregivers for individuals living with Alzheimer’s or another dementia. These percentages were applied to the estimated number of people age 18 and older in each state in July 2025, using U.S. Census Bureau data available at: <https://www.census.gov/programs-surveys/popest/data/tables.html>. This resulted in a total of 12.734 million Alzheimer’s and dementia caregivers across all 50 states and the District of Columbia.
- A8. Number of hours of unpaid care: The BRFSS survey asks caregivers to identify, within five time frames, the number of hours they provide care in an average week. Using the method developed by Rabarison and colleagues,⁵⁰³ the Alzheimer’s Association assumed the midpoint of each time frame was the average number of hours for each caregiver within that time frame and then calculated the overall average number of hours of weekly care provided by dementia caregivers in each state. This number was then converted to a yearly average and multiplied by the number of caregivers in each state⁴⁷ to determine the total number of hours of care provided. When added together, across all 50 states and the District of Columbia, the total number of hours provided by Alzheimer’s and dementia caregivers is 19.608 billion hours.
- A9. Value of unpaid caregiving: For each state, the hourly value of care was determined as the average of the state minimum hourly wage¹⁰⁵⁰ and the most recently available state median hourly cost of a home health aide. (For Nevada, the minimum wage used was the average of the minimum wage for those who are not provided health insurance and the minimum wage for those who are provided health insurance.)⁹⁶¹ The average for each state was then multiplied by the total number of hours of unpaid care in that state⁴⁸ to derive the total value of unpaid care. Adding the totals from all states and the District of Columbia resulted in an economic value of \$446.312 billion for dementia caregiving in the United States in 2025.
- A10. The 2014 Alzheimer’s Association Women and Alzheimer’s Poll: This poll questioned a nationally representative sample of 3,102 American adults about their attitudes, knowledge and experiences related to Alzheimer’s and dementia from Jan. 9, 2014, to Jan. 29, 2014. An additional 512 respondents who provided unpaid help to a relative or friend with Alzheimer’s or a related dementia were asked questions about their care provision. Random selections of telephone numbers from landline and cell phone exchanges throughout the United States

were conducted. One individual per household was selected from the landline sample, and cell phone respondents were selected if they were 18 years old or older. Interviews were administered in English and Spanish. The poll “oversampled” Hispanics/Latinos, selected from U.S. Census tracts with higher than an 8% concentration of this group. A list sample of Asian Americans was also utilized to oversample this group. A general population weight was used to adjust for number of adults in the household and telephone usage; the second stage of this weight balanced the sample to estimated U.S. population characteristics. A weight for the caregiver sample accounted for the increased likelihood of female and White respondents in the caregiver sample. Sampling weights were also created to account for the use of two supplemental list samples. The resulting interviews comprise a probability-based, nationally representative sample of U.S. adults. A caregiver was defined as an adult over age 18 who, in the past 12 months, provided unpaid care to a relative or friend age 50 or older with Alzheimer’s or another dementia. Questionnaire design and interviewing were conducted by Abt SRBI of New York.

- A11. Lewin Model on Alzheimer’s and dementia costs: These numbers come from a model created for the Alzheimer’s Association by the Lewin Group. The model estimates total payments for health care, long-term care and hospice for people with Alzheimer’s and other dementias. The model was updated by the Lewin Group in January 2015 (updating previous model) and June 2015 (addition of state-by-state Medicaid estimates). Because the model inputs use data prior to the approval of Alzheimer’s disease-modifying therapies (lecanemab and donanemab), those drug costs are not included in the initial model outputs. To address this, spending on the drugs was estimated separately based on use and cost data through the first quarter of 2025, as reported by the Centers for Medicare & Medicaid Services. (Available at <https://data.cms.gov/summary-statistics-on-use-and-payments/medicare-medicaid-spending-by-drug/medicare-quarterly-part-b-spending-by-drug>. Accessed on January 14, 2026.) Estimates were made for each payment source and then added to the Lewin model’s output of estimated 2026 spending. The 2050 projected cost estimates do not include an estimate of drug spending. The Lewin Model’s state-specific Medicaid costs for 2025 are based on an earlier estimate of state prevalence than reported here (Weuve J, Hebert LE, Scherr PA, Evans DA. Prevalence of Alzheimer disease in U.S. states. *Epidemiology* 2015;26(1):E4-6) and are inflated to 2025 dollars.
- A12. All cost estimates were inflated to year 2025 dollars using the Consumer Price Index (CPI): All cost estimates were inflated using the seasonally adjusted average prices for medical care services from all urban consumers. The relevant item within medical care services was used for each cost element. For example, the medical care item within the CPI was used to inflate total health care payments; the hospital services item within the CPI was used to inflate hospital payments; and the nursing home and adult day services item within the CPI was used to inflate nursing home payments.
- A13. Average annual per-person payments for health care and long-term care services for Medicare beneficiaries age 65 and older with and without Alzheimer’s or other dementias: Payments are unadjusted, and therefore, do not account for differences in patient characteristics, such as age or sex. Additionally, payments are based on health care utilization and payments in 2018 for the Medicare Current Beneficiary Survey and 2019 for Medicare claims data, prior to the COVID-19 pandemic, and do not reflect any post-pandemic-related changes in utilization.
- A14. Enrollment in traditional Medicare versus Medicare Advantage (Medicare Part C): Individuals eligible for Medicare can enroll in traditional Medicare, also referred to as fee-for-service Medicare and original Medicare, or Medicare Advantage, also referred to as Medicare Part C.¹⁰⁵¹ With traditional Medicare,

beneficiaries can receive care from any doctor or hospital in the United States that accepts Medicare. Generally, beneficiaries can seek care from a specialist without a referral. Traditional Medicare has fixed cost sharing, which includes coinsurance of 20% of the Medicare-approved amount for services covered by Part B after the deductible is met. Individuals enrolled in traditional Medicare can also enroll in Medicare Supplemental Insurance (also referred to as Medigap) to help cover the out-of-pocket costs. Traditional Medicare does not have an annual limit on the amount beneficiaries pay out-of-pocket. Benefits are the same for all individuals enrolled in traditional Medicare. Individuals enrolled in traditional Medicare can also enroll in a Medicare Part D plan to cover some of the costs of prescription drugs. Medicare Part D enrollment has a separate premium. With Medicare Advantage, individuals must enroll in a specific private plan. Premiums, benefits and out-of-pocket costs may vary across plans. Medicare Advantage plans have an annual limit on the amount individuals pay out-of-pocket. Individuals enrolled in a Medicare Advantage plan are not allowed to enroll in Medigap. Medicare Advantage plans are also allowed to offer additional benefits not included in traditional Medicare, such as vision, hearing and dental services as well as some non-health care benefits, such as transportation costs and gym memberships. Many Medicare Advantage plans include prescription drug coverage (Medicare Part D). Individuals enrolled in a Medicare Advantage plan have a specific network of doctors and hospitals that enrollees need to use for services to be paid by the Medicare Advantage plan. Additionally, individuals enrolled in a Medicare Advantage plan may need a referral to see a specialist. Enrollment in Medicare Advantage has increased dramatically over the past decade, with 51% of all Medicare beneficiaries enrolled in a Medicare Advantage plan in 2025 compared with 29% in 2013.⁹⁴⁰

- A15. Medicare Current Beneficiary Survey Report: These data come from an analysis of findings from the 2018 Medicare Current Beneficiary Survey (MCBS). The analysis was conducted for the Alzheimer’s Association by Health Care Cost Institute.⁹⁰⁷ The MCBS, a continuous survey of a nationally representative sample of about 15,000 Medicare beneficiaries, is linked to Medicare claims. The survey is supported by the U.S. Centers for Medicare & Medicaid Services (CMS). For community-dwelling survey participants, MCBS interviews are conducted in person three times a year with the Medicare beneficiary or a proxy respondent if the beneficiary is not able to respond. For survey participants who are living in a nursing home or another residential care setting, such as an assisted living residence, retirement home or a long-term care unit in a hospital or mental health facility, MCBS interviews are conducted with a staff member designated by the facility administrator as the most appropriate to answer the questions. Data from the MCBS analysis that are included in *2025 Alzheimer’s Disease Facts and Figures* pertain only to Medicare beneficiaries age 65 and older.

For this MCBS analysis, people with dementia are defined as:

- Community-dwelling survey participants who answered yes to the MCBS question, “Has a doctor ever told you that you had Alzheimer’s disease or dementia?” Proxy responses to this question were accepted.
- Survey participants who were living in a nursing home or other residential care setting and had a diagnosis of Alzheimer’s disease or dementia in their medical record.
- Survey participants who had at least one Medicare claim with a diagnostic code for Alzheimer’s or other dementias in 2018. The claim could be for any Medicare service, including hospital, skilled nursing facility, outpatient medical care, home health care, hospice or physician, or other health care provider visit. The diagnostic codes used to identify survey participants with Alzheimer’s or other dementias are G30.0, G30.1, G30.8, G30.9, G31.01, G31.09, G31.83, F01.50, F01.51, F02.80, F02.81, F03.90, F03.91 and F10.27.

Costs from the MCBS analysis are based on responses from 2018 and reported in 2025 dollars.

A16. Differences in estimated costs reported by Hurd and colleagues:
Hurd and colleagues⁹⁰⁶ estimated per-person costs using data from participants in ADAMS, a cohort in which all individuals underwent diagnostic assessments for dementia. One reason that the per-person costs estimated by Hurd and colleagues are lower than those reported in *Facts and Figures* is that ADAMS, with its diagnostic evaluations of everyone in the study, is more likely than MCBS to have identified individuals with less severe or undiagnosed Alzheimer's. By contrast, the individuals with Alzheimer's registered by MCBS are likely to be those with more severe, and therefore more costly, illness. A second reason is that the Hurd et al. estimated costs reflect an effort to isolate the incremental costs associated with Alzheimer's and other dementias (those costs attributed only to dementia), while the per-person costs in *2026 Alzheimer's Disease Facts and Figures* incorporate all costs of caring for people with the disease (regardless of whether the expenditure was related to dementia or a coexisting condition).

Appendices

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